

Document analysis report

DOCUMENT ANALYSIS

Document	Example: human essay (example dataset, CC BY 3.0)
Date	2026-10-02 23:46:17
Word count	578
Language	en
Model	en-20261002-783106e4aa39-nonnative
Privacy	Privacy mode: Local processing. No text leaves this computer.

AI-text detection is probabilistic. The results should not be used as sole evidence for academic misconduct, disciplinary action, or authorship determination. The detector is designed to assist human review, not to replace it.

RESULT

AI-associated probability (share of text)	6%
Human-associated probability (share of text)	26%
Uncertain / mixed	68%
Document-level AI probability (calibrated)	1%
Category	Human-assisted

Based on the characteristics measured by this model, the text contains patterns that are not notably associated with AI-generated writing. This is a statistical estimate, not a determination of authorship.

Shares are fractions of words in sentences whose calibrated sentence probability is ≥ 0.73 (AI-associated), ≤ 0.18 (human-associated) or in between (uncertain/mixed).

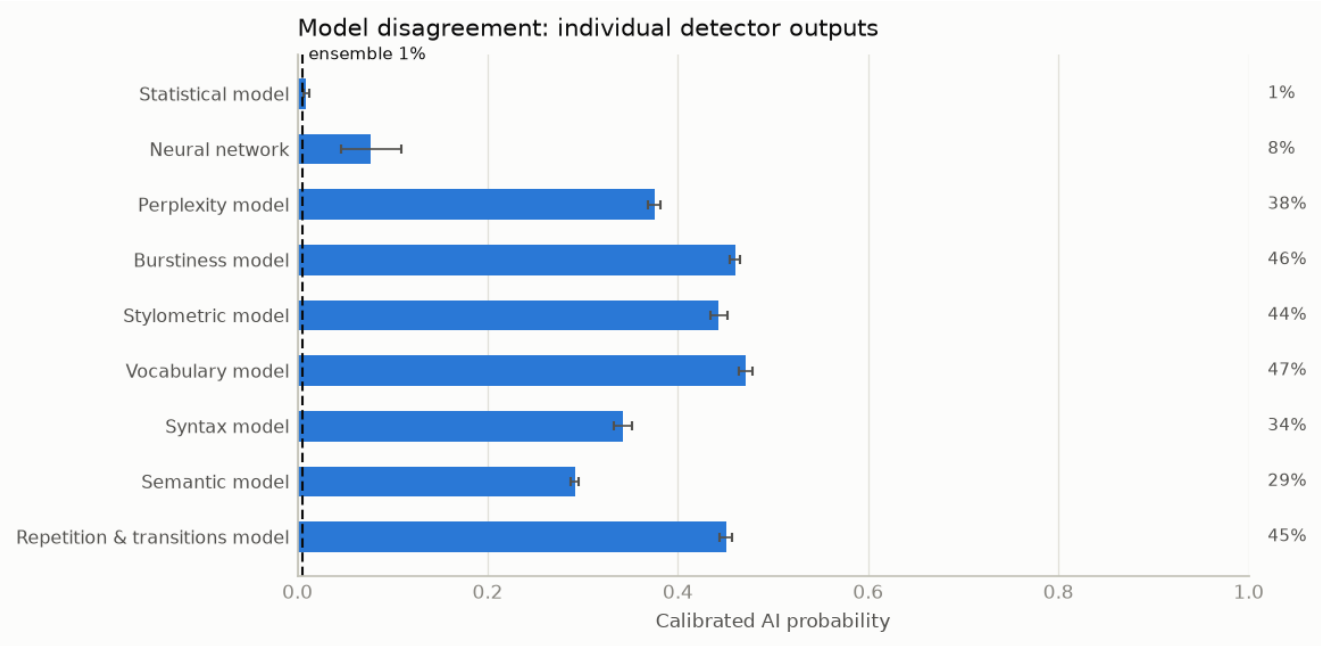
CONFIDENCE

High (95%) - More reliable statistical analysis (text length). Confidence is separate from likelihood: it reflects text length, detector agreement, feature coverage and committee stability.

length reliability	0.99
detector agreement	0.91
feature coverage	1.00
committee stability	0.98

MODEL DISAGREEMENT

The detectors partly disagree.



Detector	AI prob.	± error	weight
Statistical model (gradient boosting, all features)	1%	0.003	0.65
Neural network (MLP, all features)	8%	0.032	0.35
Perplexity model	38%	0.006	0.00
Burstiness model	46%	0.006	0.00
Stylometric model	44%	0.009	0.00
Vocabulary model	47%	0.007	0.00
Syntax model	34%	0.010	0.00
Semantic model	29%	0.004	0.00
Repetition & transitions model	45%	0.007	0.00

SIGNALS

Strongest signals toward AI-associated patterns (the overall score is not elevated):

- many sequencing transitions
- rarer, more sophisticated vocabulary
- less predictable word choices (higher perplexity)
- few unknown words or misspellings
- many commas

These are statistical indicators, not proof of AI authorship.

STATISTICAL ANALYSIS

Perplexity

Measure	Value	Human pct.	Human median	AI median
Mean sentence surprisal (bits/word (unigram proxy))	12.161	83.700	11.184	11.362
Median word surprisal	12.756	89.300	11.295	11.760
Lowest sentence surprisal	8.971	29.300	9.464	9.899

Burstiness

Measure	Value	Human pct.	Human median	AI median
Sentence perplexity std.	1.653	87.900	1.040	0.899

Perplexity burstiness index B	-0.761	85.300	-0.829	-0.853
Sentence-length variation (CV)	0.349	26.900	0.435	0.326
Sentence-length burstiness B	-0.482	26.900	-0.394	-0.508
Syntactic variation (CV of depth)	0.432	72.600	0.363	0.312
Stylometry				
Measure	Value	Human pct.	Human median	AI median
Average sentence length (words)	19.3	45.000	20.000	19.887
Median sentence length	19.0	50.000	19.000	19.500
Average word length	5.62	94.500	4.863	5.073
Function-word share	0.409	25.000	0.453	0.435
Contractions per 100 words	0.00	55.000	0.000	0.000
First-person singular share	0.0000	55.000	0.000	0.000
Adverb share	0.029	31.600	0.040	0.031
Adjective share	0.138	93.100	0.083	0.091
Punctuation				
Measure	Value	Human pct.	Human median	AI median
Commas per 100 words	7.79	91.100	4.647	4.799
Semicolons per 100 words	0.17	85.200	0.000	0.000
Colons per 100 words	0.35	78.500	0.000	0.000
Dashes per 100 words	0.00	75.000	0.000	0.000
Parentheses per 100 words	1.38	80.500	0.461	0.000
Punctuation variety (entropy)	0.462	46.900	0.471	0.386
Vocabulary				
Measure	Value	Human pct.	Human median	AI median
Moving-average type-token ratio	0.808	61.600	0.796	0.794
MTLD lexical diversity	99.0	76.900	79.115	76.000
Hapax legomena ratio	0.813	81.600	0.776	0.774
Word commonness (Zipf)	4.13	4.800	4.642	4.581
Rare-word share	0.183	95.400	0.052	0.044
Vocabulary growth (Heaps)	0.809	49.900	0.809	0.801
Syntax				
Measure	Value	Human pct.	Human median	AI median
Dependency-tree depth	5.90	56.300	5.700	6.000
Clauses per sentence	2.07	30.000	2.417	2.350
Subordinate clauses per sentence	0.77	31.700	1.000	0.933
Passive-voice sentences	0.467	91.200	0.174	0.150
Sentence-opening variety	0.747	92.700	0.581	0.525
Syntactic diversity	0.630	17.600	0.706	0.661
Repetition				
Measure	Value	Human pct.	Human median	AI median
Repeated 3-word phrases	0.014	38.200	0.021	0.036

Repeated grammatical templates	0.581	87.400	0.472	0.532
Repeated sentence structures	0.042	35.800	0.125	0.188
Transition words per sentence	0.300	86.500	0.100	0.107
Transition diversity	0.889	45.600	1.000	1.000
Sentence-initial transitions	0.889	56.100	0.750	0.800

Semantic

Measure	Value	Human pct.	Human median	AI median
Neighbouring-sentence similarity	0.099	39.000	0.116	0.133
Local coherence gain	0.039	63.500	0.027	0.002
Topic-jump rate	0.483	40.700	0.500	0.571
Semantic redundancy	0.210	27.500	0.258	0.302
Semantic compression	0.064	8.900	0.129	0.151

Consistency

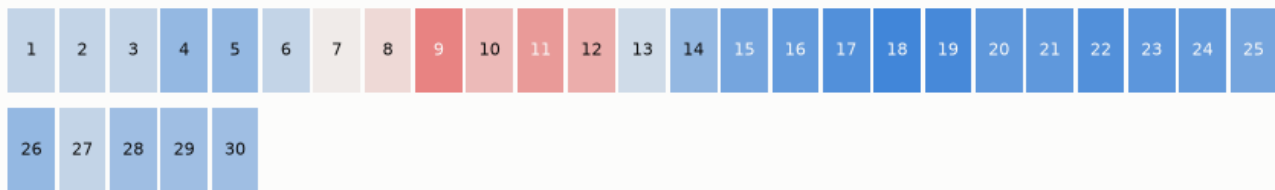
Measure	Value	Human pct.	Human median	AI median
Style drift (mean JS divergence)	0.017	37.900	0.019	0.014
Largest style shift	0.031	44.100	0.033	0.026

Paragraphs

Measure	Value	Human pct.	Human median	AI median
Paragraph-length variation (CV)	0.246	n/a	n/a	n/a
Paragraph perplexity variation	0.596	n/a	n/a	n/a

HEATMAP

AI-likelihood heatmap (one cell per sentence)



¹ Allergic rhinitis is an intermittent or persistent inflammation of the mucous membrane of the upper respiratory tract, characterized by nasal congestion, discharge, itching, sneezing, and a combination of several symptoms is possible. ² In this situation, a 35-year-old patient presented with nasal congestion, eyelid edema, conjunctival erythema, allergic folds and spangles, and inflamed nostrils, which confirms her diagnosis (Seidman et al. 2015). ³ A medical specialist identified some of the symptoms listed.

⁴ The hypersensitivity reaction is the first type since allergic rhinitis is determined by it. ⁵ The allergen, having entered the body for the first time, causes the appearance of specific antibodies, immunoglobulins, which are fixed on the surface of mast cells. ⁶ Upon repeated contact of an already sensitized organism with an irritant, IgE-dependent activation of mast cells occurs (Hoyte & Nelson, 2018). ⁷ This process causes the release of histamine, heparin, and other inflammatory mediators responsible for the manifestation of allergic reactions. ⁸ In fact, pathophysiology implies that the initial intake of the allergen causes the production of IgE by plasma cells.

⁹ Three subjective conclusions from this situation are: the patient experiences constant sneezing, considers the complication of infectious diseases and describes improvement away from home in a different climate. ¹⁰ A specialist established objective conclusions: firstly, eyelid edema and allergic sheen caused by venous edema of the lower eyelid. ¹¹ In other words, allergies give inflammation to the eyes. ¹² Secondly, inflamed nostrils and an allergic fold indicate a stable runny nose. ¹³ Thirdly, the constancy of these signs shows the constant close presence of a source of irritation.

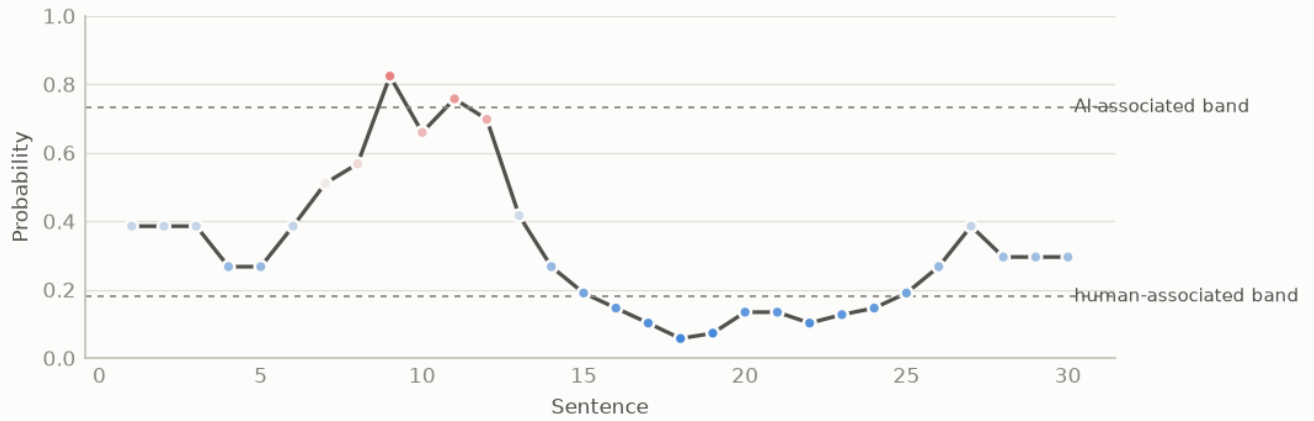
¹⁴ First of all, it is necessary to pass tests for sources of irritation, which are taken by the method of a swab from the nose. ¹⁵ Based on the findings, pharmacological treatment with antihistamines and corticosteroid nasal sprays and decongestants is then prescribed to reduce congestion and symptoms (Hoyte & Nelson, 2018). ¹⁶ Equally important are environmental control and nasal irrigation (Schuler IV & Montejó, 2021). ¹⁷ Finally, as a consequence, as little contact with known allergens as possible should be ensured. ¹⁸ In this case, the symptoms appeared around the same time as the cat appeared in the house. ¹⁹ Since humans often show hypersensitivity to animal saliva, this option is very likely to be used as a hypothesis.

²⁰ Thus, the following approach is required. ²¹ First of all, pharmacological treatment with intranasal antihistamines and reduction of contact with the cat is necessary if the source of hypersensitivity is confirmed (Seidman et al. 2015). ²² If the proposed treatment is ineffective, it is necessary to resort to more complex pharmacological treatment or surgical interventions in the nasal area (Seidman et al. 2015). ²³ Antihistamine class of drugs and decongestants are offered as pharmacological treatment.

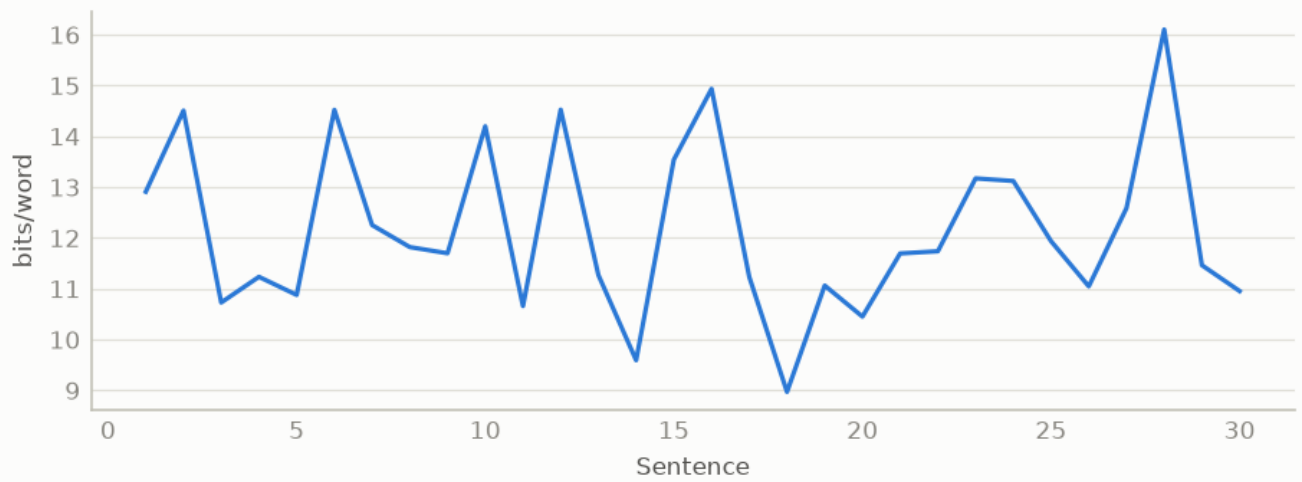
²⁴ Fexofenadine is the pharmacologically active metabolite of terfenadine; it does not have a sedative effect. ²⁵ The antihistamine effect appears after 1 hour, reaching a maximum after 6 hours, lasting for 24 hours. ²⁶ After 28 days of the administration, no addiction is observed, which is an achievement compared to similar drugs of the previous generation (Klimek et al. 2019). ²⁷ Phenylephrine is presented as a part of combined preparations for treating acute respiratory viral infections, and influenza is used to achieve a moderate vasoconstrictive effect. ²⁸ Reduces swelling and hyperemia of the nasal mucosa by stimulating α 1-adrenergic receptors (Klimek et al. 2019). ²⁹ In this situation, the abuse of intranasal vasoconstrictor drugs, which can be addictive and only eliminate symptoms, is not recommended. ³⁰ It is also not recommended to wear masks and other personal protective equipment that restricts the respiratory system.

GRAPHS

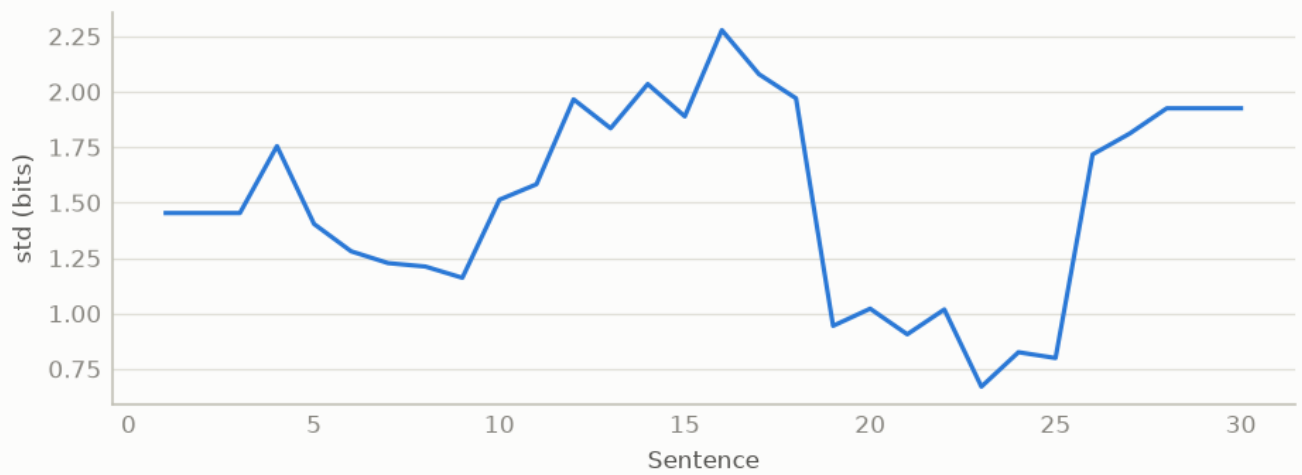
Sentence AI likelihood (calibrated)



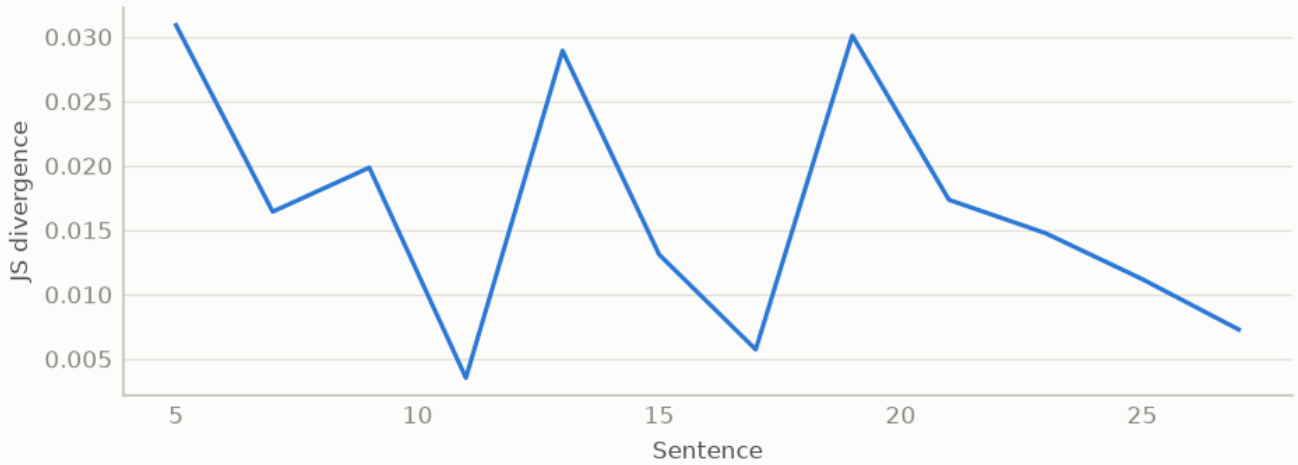
Sentence perplexity (mean surprisal)



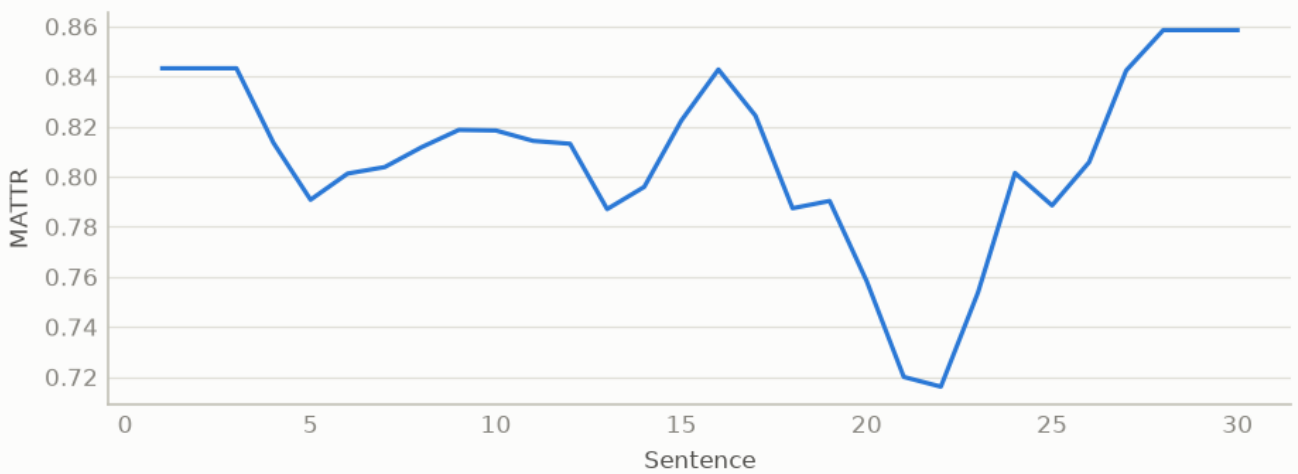
Burstiness: local variation of predictability



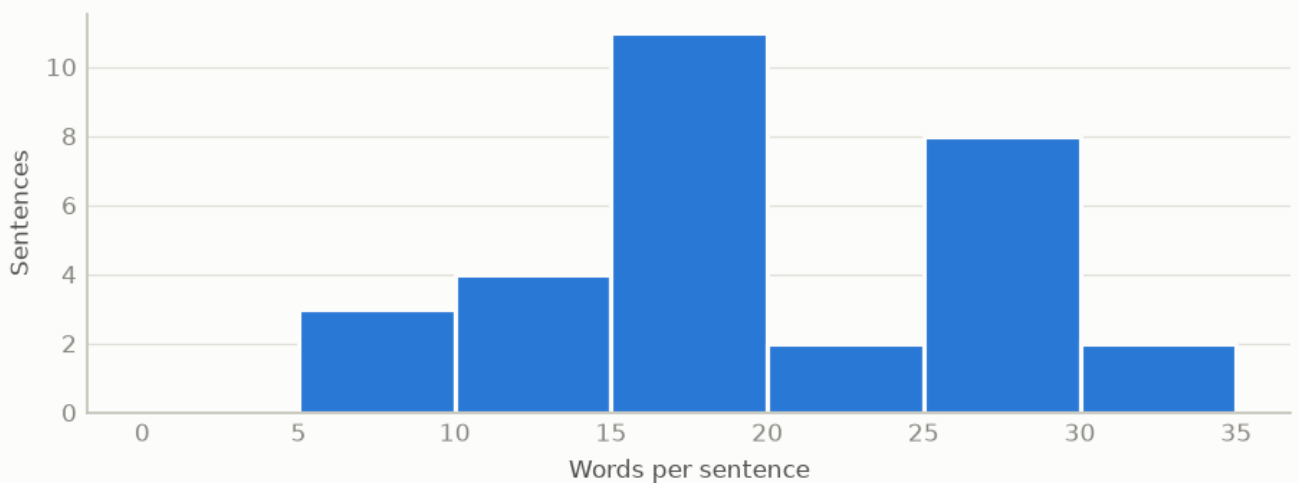
Style drift between consecutive windows



Vocabulary diversity (MATTR, 5-sentence window)



Sentence-length distribution



POTENTIAL AUTHORSHIP/STYLE TRANSITIONS

No statistically significant style transition was detected.

SECTION ANALYSIS (sentence by sentence)

#	Sentence	AI lik.	Conf.
1	Allergic rhinitis is an intermittent or persistent inflammation of the mucous membrane of the upper respiratory tract, characterized by nasal congestion, discharge, itching, sneezing, and a combination of several symptoms is possible.	39%	High
2	In this situation, a 35-year-old patient presented with nasal congestion, eyelid edema, conjunctival erythema, allergic folds and spangles, and inflamed nostrils, which confirms her diagnosis (Seidman et al. 2015).	39%	High
3	A medical specialist identified some of the symptoms listed.	39%	High
4	The hypersensitivity reaction is the first type since allergic rhinitis is determined by it.	27%	High
5	The allergen, having entered the body for the first time, causes the appearance of specific antibodies, immunoglobulins, which are fixed on the surface of mast cells.	27%	High
6	Upon repeated contact of an already sensitized organism with an irritant, IgE-dependent activation of mast cells occurs (Hoyte & Nelson, 2018).	39%	High
7	This process causes the release of histamine, heparin, and other inflammatory mediators responsible for the manifestation of allergic reactions.	51%	High
8	In fact, pathophysiology implies that the initial intake of the allergen causes the production of IgE by plasma cells.	57%	High
9	Three subjective conclusions from this situation are: the patient experiences constant sneezing, considers the complication of infectious diseases and describes improvement away from home in a different climate.	82%	Medium
10	A specialist established objective conclusions: firstly, eyelid edema and allergic sheen caused by venous edema of the lower eyelid.	66%	High
11	In other words, allergies give inflammation to the eyes.	76%	High
12	Secondly, inflamed nostrils and an allergic fold indicate a stable runny nose.	70%	Medium
13	Thirdly, the constancy of these signs shows the constant close presence of a source of irritation.	42%	High
14	First of all, it is necessary to pass tests for sources of irritation, which are taken by the method of a swab from the nose.	27%	High
15	Based on the findings, pharmacological treatment with antihistamines and corticosteroid nasal sprays and decongestants is then prescribed to reduce congestion and symptoms (Hoyte & Nelson, 2018).	19%	High
16	Equally important are environmental control and nasal irrigation (Schuler IV & Montejo, 2021).	15%	High
17	Finally, as a consequence, as little contact with known allergens as possible should be ensured.	10%	Medium
18	In this case, the symptoms appeared around the same time as the cat appeared in the house.	6%	Medium
19	Since humans often show hypersensitivity to animal saliva, this option is very likely to be used as a hypothesis.	7%	High
20	Thus, the following approach is required.	13%	High
21	First of all, pharmacological treatment with intranasal antihistamines and reduction of contact with the cat is necessary if the source of hypersensitivity is confirmed (Seidman et al. 2015).	13%	High
22	If the proposed treatment is ineffective, it is necessary to resort to more complex pharmacological treatment or surgical interventions in the nasal area (Seidman et al. 2015).	10%	Medium
23	Antihistamine class of drugs and decongestants are offered as pharmacological treatment.	13%	High
24	Fexofenadine is the pharmacologically active metabolite of terfenadine; it does not have a sedative effect.	15%	High
25	The antihistamine effect appears after 1 hour, reaching a maximum after 6 hours, lasting for 24 hours.	19%	High
26	After 28 days of the administration, no addiction is observed, which is an achievement compared to similar drugs of the previous generation (Klimek et al. 2019).	27%	High
27	Phenylephrine is presented as a part of combined preparations for treating acute respiratory viral infections, and influenza is used to achieve a moderate vasoconstrictive effect.	39%	High
28	Reduces swelling and hyperemia of the nasal mucosa by stimulating α 1-adrenergic receptors (Klimek et al. 2019).	30%	High
29	In this situation, the abuse of intranasal vasoconstrictor drugs, which can be addictive and only eliminate symptoms, is not recommended.	30%	High
30	It is also not recommended to wear masks and other personal protective equipment that restricts the respiratory system.	30%	High

LIMITATIONS

- AI-text detection is probabilistic and produces both false positives (human text flagged) and false negatives (AI text missed).
- Probabilities are calibrated on held-out data with equal numbers of human and AI documents (a 50% prior). The share of AI text in your context changes how a given score should be read.
- Performance drops on text from AI models, domains, languages and writing populations that differ from the training data; see the evaluation report for measured cross-model and cross-domain results.
- Paraphrasing, translation, heavy human editing and short texts reduce detection reliability.
- Formulaic genres (legal, scientific, technical) and highly polished human writing can resemble AI-generated text statistically.
- Spelling or grammar mistakes, dialect, nationality and writing ability are never used as evidence of AI authorship, but non-native writing can still differ statistically from the training data; check the false-positive audit in the evaluation report.
- Sentence-level scores use a 5-sentence context window and are less reliable than document-level scores.
- Detected style transitions indicate a statistical change in writing characteristics, not a change of author.

AI-text detection is probabilistic. The results should not be used as sole evidence for academic misconduct, disciplinary action, or authorship determination. The detector is designed to assist human review, not to replace it.